

The looksmaxxing landscape and how to read it

Week 1, Lecture 2 · Looksmaxxing

Autumn 2026 · A Ten-Week Evidence-Based Self-Improvement Course

What we'll cover

- What looksmaxxing communities actually advocate
- Soft vs hardmaxxing: where each stands on the evidence
- An evidence hierarchy you can use on any claim
- Why forum epistemology inverts the burden of proof
- Bonesmashing: the evidence picture
- Mewing: what the AAO found when it looked

Soft vs hardmaxxing

The landscape in two camps

Softmaxxing targets behavioral and habit-based interventions:

- Skincare and sunscreen
- Sleep, nutrition, and exercise
- Grooming, posture, and style
- Evidence base: strong to moderate across multiple interventions

Hardmaxxing targets structural change:

- Surgical procedures (implants, rhinoplasty, LeFort osteotomy)
- Bone remodeling via mechanical force (bonesmashing)
- Skeletal repositioning via tongue posture (mewing)
- Evidence base: ranges from good (surgery under clinical protocols) to absent (bonesmashing) to zero (mewing in adults)

This course covers softmaxxing exclusively.

Why the distinction matters

Hardmaxxing rhetoric often bleeds into softmaxxing spaces online. A post about moisturizer sits next to a post about orbital bone reshaping as if both are the same kind of advice.

They are not the same kind of advice:

- One has a risk/benefit profile you can evaluate from published data
- One asks you to harm yourself on the basis of a forum poster's theory

The evidence hierarchy in the next section is the tool for separating them.

The evidence hierarchy

Ranking evidence for self-improvement claims

From strongest to weakest:

1. **Systematic review or meta-analysis** of controlled trials
2. **Individual randomized controlled trial (RCT)**
3. **Observational study** (cohort, case-control)
4. **Professional guideline** from a recognized clinical body
5. **Mechanistic argument** supported by basic science
6. **Case report or anecdote**
7. **Absence of disproof** ("no one has proved it wrong")

Claims from looksmaxxing forums almost always sit at levels 6 and 7.

The reversed burden of proof

Level 7 is the error at the center of most hardmaxxing claims:

"There is no peer-reviewed evidence against bonesmashing, therefore it works."

This is not a weak argument. It is a structurally invalid argument.

The burden of proof sits with the person making the positive claim. "No one has proved it doesn't work" proves nothing about whether it does work. It proves only that it has not been studied.

Every untested practice in medicine history would pass this test. That is why it is not evidence.

How to apply the hierarchy

Three questions to ask of any claim:

1. What level of evidence is the poster citing?
2. If they cite no evidence, are they offering a mechanism or only an anecdote?
3. If they invoke level 7 (absence of disproof), identify the error explicitly.

A claim can move up the hierarchy if evidence is gathered. "Suggestive" is not the same as "unfounded." The hierarchy helps you hold both at once.

Case study: bonesmashing

What bonesmashing claims

The practice involves striking facial bones (typically the orbital rim, cheekbones, or jaw) with a hard object to produce micro-fractures intended to stimulate periosteal bone remodeling.

The claimed mechanism: mechanical stress triggers bone remodeling in the direction of the force, producing a harder, more defined facial structure over time.

Appealing to the poster because it has a biological-sounding mechanism at level 5.

The evidence picture for bonesmashing

The mechanism (bone remodels in response to mechanical stress) is established in orthopedics for controlled loading contexts: weight-bearing exercise, orthodontic appliances, prosthetics.

What is not established:

- That random blunt trauma to the face produces directional remodeling rather than damage
- That the forces involved are in the therapeutic range rather than the injury range
- That the claimed outcomes (wider cheekbones, more defined jaw) have occurred in any documented case

The looksmax.org thread (2022) presents the absence of studies disconfirming bonesmashing as positive evidence. That is level 7 reasoning.

Label: **Unfounded**. The mechanism is suggestive; the practice is not supported.

Case study: mewing

What mewing claims

Mewing is tongue posture practice popularized by Dr. John (Mike) Mew: resting the tongue against the palate is claimed to expand the palate, advance the midface, and improve jaw definition over time.

The origin: Mike Mew extrapolated from his father Dr. John Mew's orthotropic treatment approach, which is itself contested within orthodontics.

What the AAO found

The American Association of Orthodontists (2023) reviewed the available literature on mewing:

- Zero peer-reviewed studies support mewing's effectiveness for reshaping the jaw or bite in adults
- Mike Mew was struck from the UK dental register in 2024
- The AAO does not endorse mewing as a treatment for any condition

Two claims need to be kept separate:

- Palate expansion in growing children (some evidence; not the same as adult mewing)
- Structural bone change in skeletally mature adults from tongue posture (no evidence)

Why mewing spreads despite zero evidence

Three amplification mechanisms:

1. **Before-and-after photos** with no controls, no blinded raters, and confounding from weight loss and lighting
2. **Anecdotal confirmation bias:** people who believe it worked notice supportive moments; negative results are not posted
3. **Unfalsifiable framing:** "you need to do it for years before seeing results" immunizes the claim against short-term tests

None of these are arguments for efficacy. Recognizing them is a skill.

"The American Association of Orthodontists has extensively reviewed available literature and found zero peer-reviewed studies supporting mewing's effectiveness for reshaping the jaw or bite."

American Association of Orthodontists (2023)

Does Mewing Actually Reshape Your Jaw?

A note on obsession

Looksmaxxing communities attract people at risk for appearance preoccupation. That is worth naming directly.

Spending hours daily on forum research about bone structure you cannot change is a warning sign, not a self-improvement strategy. The warning signs to watch:

- Researching appearance for more than an hour a day
- Significant distress from perceived flaws others do not notice
- Avoidance of social situations because of appearance concerns
- Repeatedly seeking reassurance about appearance

The International OCD Foundation has a BDD resource hub at bdd.iocdf.org. Week 10 returns to this topic at length.

Take-aways

1. Softmaxxing targets behavioral change with moderate to strong evidence. Hardmaxxing ranges from clinical surgery (which has an evidence base) to practices with no evidence at all.
2. The evidence hierarchy lets you rank any claim without being a scientist. Start by asking what level the poster is citing.
3. Absence of disproof is not evidence. The burden of proof sits with the person making the positive claim.
4. Bonesmashing has a plausible mechanism and zero evidence of efficacy: label it unfounded. Mewing in adults has zero peer-reviewed support per the AAO (2023): label it unfounded.
5. Appearance obsession is a real risk in looksmaxxing communities. If you notice the warning signs, the resources exist and work.

What's next

- **Reading:** “Attractiveness research and what you can actually change” at </c/looksmaxxing-26au/readings/wk01>
- **Section this week:** Annotate three forum posts using the evidence hierarchy you just learned
- **HW1 (out this week):** Honest baseline audit
- **Week 2:** Baseline measurement, sleep, and building the habit machine for the next eight weeks